

TENTATIVE RULING(S) FOR August 27, 2026

Department S37 – Judge Winston Keh

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CIVSB2406055

Padilla vs. Del Rosa Villaidence et al (2 of 2)

TENTATIVE RULING

I. FACTUAL AND PROCEDURAL BACKGROUND

On February 20, 2024, Plaintiff Jose Sencion Padilla filed this action against Defendants Del Rosa Villa, LLC (dismissed); Del Rosa Villaidence Opco, LLC (Defendant or “Del Rosa”); Providence Group of Southern California, LLC (dismissed); Providence Group, Inc. (dismissed); and 2018 N. Del Rosa Avenue Propco, LLC (dismissed). Plaintiff asserts causes of action for (1) elder abuse and neglect, (2) statutory violations of the Patient Bill of Rights, and (3) wrongful death.

Plaintiff, on behalf of decedent Maria Sencion, alleges that between September 4, 2022 and September 6, 2022, Defendants failed to provide adequate staffing, appropriate diet and feeding

assistance, monitoring for choking, and timely medical intervention, and failed to follow physician orders and document changes in Ms. Sencion's condition. (Compl. ¶¶ 12-23.) Plaintiff alleges that, as a result of these omissions, Ms. Sencion was left unattended, given improper food, and ultimately choked to death. (Compl. ¶¶ 24-27.)

Del Rosa now moves for summary judgment or, alternatively, summary adjudication of each cause of action and Plaintiff's prayer for punitive damages.

The motion was originally scheduled for hearing on May 4, 2026. Plaintiff thereafter applied ex parte for a continuance to permit additional discovery, including depositions Plaintiff contended were necessary to oppose the motion. On April 30, 2026, the Court granted the application, took the May 4 hearing off calendar, and continued the motion to August 4, 2026. The motion was subsequently reset to August 27, 2026. Plaintiff opposes and Defendant replies.

II. EVIDENTIARY OBJECTIONS

A. Plaintiff's Objections to Lyndsay Sklenka's Declaration

Plaintiff objects to Sklenka's authentication of Exhibits A through E and separately objects to the Del Rosa, AMR, and hospital records as hearsay lacking a business-records foundation. Plaintiff also objects derivatively to material facts and expert opinions that rely upon those records. (Pl.'s Evid. Obj. Nos. 1-3.)

Although Sklenka is Defendant's counsel rather than a records custodian, the supplemental record establishes that Defendant produced the Del Rosa, St. Bernardine, and AMR records to Plaintiff in discovery, and Plaintiff independently submitted and relied upon those same records in opposing the motion. (Apodaca Decl. ¶¶ 13, 16-17.)

Plaintiff relies on *Garibay v. Hemmat* (2008) 161 Cal.App.4th 735 (*Garibay*), where the defendant's expert relied upon medical records and an operative note that were not in evidence or otherwise properly before the trial court. The Court of Appeal held that, absent those records and an adequate evidentiary foundation for the facts upon which the expert relied, the defendant failed to satisfy its initial burden on summary judgment. (*Id.* at pp. 742-743.)

The circumstances here are materially different. The records before the Court were produced by Defendant during discovery, and were submitted and relied upon by Plaintiff. Plaintiff therefore

does not meaningfully dispute that these are the records produced in this litigation. Moreover, Plaintiff's generalized hearsay objection does not identify particular statements within the records that lack an applicable exception or otherwise require exclusion.

Accordingly, *Garibay* does not require exclusion of the records or of Dr. Steinke's opinions based upon them. An expert may rely upon materials reasonably considered in forming an opinion, although that reliance does not independently establish the admissibility for their truth of every case-specific statement contained within those materials.

The Court OVERRULES Plaintiff's Evidentiary Objections Nos. 1-3.

B. Plaintiff's Objections to Dr. Gary W. Steinke's Declaration

Plaintiff's amended objections principally challenge Dr. Steinke's qualifications and foundation, his reliance on medical records, and the reasoning supporting his standard-of-care and causation opinions.

Dr. Steinke is a licensed physician who establishes experience with geriatric patients and skilled nursing facilities and sufficient familiarity with the applicable standard of care. The challenges to the factual basis and persuasiveness of his opinions generally concern the weight to be afforded the opinions rather than their admissibility.

The Court OVERRULES Objections Nos. 1-7 and 9.

As to Objection No. 8, the Court **SUSTAINS** the objection to Dr. Steinke's statement in paragraph 29 that there is "no merit whatsoever" to Plaintiff's elder abuse and neglect claims. Whether the evidence establishes a statutory cause of action for elder abuse or neglect is a legal determination for the Court and, where appropriate, the trier of fact. It is not a proper subject for expert opinion.

As to Objection No. 10, the Court **SUSTAINS** the objection to Dr. Steinke's opinion in paragraph 31 that Defendant did not violate Health and Safety Code section 1430 or applicable regulatory standards. Dr. Steinke may testify concerning the applicable standard of care and the care provided to Ms. Sencion, but he may not offer a legal conclusion concerning whether Defendant violated a statute or regulation.

C. Defendant's Objections to George Barnes's First Amended Declaration

Defendant asserts 25 objections to the First Amended Declaration of Plaintiff's expert, George Barnes, MS, CCC-SLP, BCS-S. Although Defendant's July 24, 2026 filing is captioned as objections to the "Declaration of George Barnes," the objections are directed to Barnes's First Amended Declaration filed July 15, 2026, as demonstrated by Objections Nos. 23-25, which address opinions newly added in paragraphs 45-47.

The objections also contain several paragraph-numbering errors. The Court therefore rules upon the numbered objections and the testimony actually challenged, rather than the paragraph numbers stated in Defendant's filing.

The First Amended Declaration supplies greater foundation than the declaration previously before the Court. Barnes is a licensed speech language pathologist, is board certified in swallowing and swallowing disorders, has more than a decade of clinical experience involving dysphagia and medically complex patients, and has worked in subacute care, acute rehabilitation, critical illness recovery, and interdisciplinary clinical settings. (Barnes Am. Decl. ¶¶ 6-7, 10, 12-15.) His curriculum vitae is attached.

The Court finds that Barnes possesses sufficient expertise to offer opinions concerning dysphagia, swallowing function, aspiration and choking risk, feeding safety, and related dysphagia management.

Objection No. 1: OVERRULED. Barnes establishes sufficient education, training, certification, and experience to testify within his identified areas of expertise.

Objection No. 2: SUSTAINED. Barnes's statement that he has never had his opinions excluded or been excluded or limited as an expert witness is not relevant to whether his opinions in this case are admissible.

Objections Nos. 3-11: OVERRULED. The challenged opinions concern dysphagia, airway compromise, and the education, certification, clinical experience, and professional background supporting Barnes's expertise.

Objections Nos. 12-15: OVERRULED. The challenged opinions concern whether Ms. Sencion experienced dysphagia-related choking, whether the identified risks warranted an SLP

consultation or swallowing evaluation, and whether such an evaluation could have been pursued despite her COVID-19 status. These subjects fall within Barnes's demonstrated expertise.

Objection No. 16: SUSTAINED IN PART. Barnes may testify concerning safe feeding practices, recognition of dysphagia and aspiration, and individualized feeding strategies. The objection is sustained as to the statements: "Ms. Olivia Hall's admitted lack of training since 1998 is an egregious violation of these CMS and DHHS requirements" and "This reveals an institutional disregard for the essential training standards required to protect resident health and safety at Del Rosa Villa—a failure originating from the highest levels of management." The first statement constitutes an improper legal conclusion, and the second attributes an institutional state of mind beyond Barnes's clinical expertise. The objection is otherwise overruled.

Objections Nos. 17-18: OVERRULED. Barnes may explain the clinical significance of communication, documentation, and medical history to the assessment and management of dysphagia and swallowing risk.

Objection No. 19: SUSTAINED IN PART. Barnes may testify concerning the clinical importance of obtaining a patient's baseline swallowing and choking history. The objection is sustained as to the phrase "making it a regulatory standard" and the statements: "Either Mrs. Sencion's swallowing issues came up during these discussions, and it was never documented and never addressed or the discussion never took place. In both of these scenarios, it shows the team at Del Rosa Villa acted below the standard of care." The former is an improper legal conclusion; the latter is speculative because Barnes lacks personal knowledge concerning which event occurred. The objection is otherwise overruled.

Objection No. 20: OVERRULED. Barnes identifies specific discrepancies between the accounts and the contemporaneous AMR and hospital records.

Objection No. 21: SUSTAINED IN PART. Barnes may compare the original and subsequent entries and explain the clinical significance of the differences. The objection is sustained only as to the phrase "seemingly to change the way the incident is perceived," because Barnes lacks personal knowledge concerning the purpose or intent behind any modification. The objection is otherwise overruled.

Objection No. 22: SUSTAINED IN PART. Barnes may explain signs of dysphagia, circumstances warranting an SLP referral or swallowing evaluation, and appropriate feeding precautions. The objection is sustained only as to the statement: “This underscores a facility culture that prioritized minimal training over individual patient safety.” That statement improperly attributes an institutional state of mind beyond Barnes’s clinical expertise. The objection is otherwise overruled.

Objection No. 23: OVERRULED. Barnes’s expertise in dysphagia, swallowing, airway protection, and choking, together with his clinical experience with medically complex patients, provides sufficient foundation for the challenged opinions concerning recognition and response to a choking event.

Objection No. 24: SUSTAINED. Barnes’s opinion that the care provided represented a “systemic breakdown of basic support” extends beyond his demonstrated expertise in dysphagia and swallowing care.

Objection No. 25: SUSTAINED IN PART. Barnes may testify concerning the texture and consistency of the food served and whether chopped rather than ground chicken was consistent with the prescribed soft diet. The objection is sustained as to the statements: “Ms. Sencion was also not supposed to be provided with potatoes, being on a renal diet” and “The inclusion of at least two food items inconsistent with her dietary orders strongly suggests that Ms. Sencion was served the wrong tray for the lunch meal during which she choked.” Barnes has not established expertise in renal nutrition. The objection is otherwise overruled.

D. Defendant’s Objections to the Declarations of Ruben Sencion and Olga Sencion

Defendant previously objected to the Declarations of Ruben Sencion and Olga Sencion submitted with Plaintiff’s original opposition. Plaintiff relies upon the same declarations and testimony in the First Amended Opposition and Amended Separate Statement.

Defendant’s objection to the Declaration of Ruben Sencion is **OVERRULED**. His material testimony concerning Ms. Sencion’s diet, swallowing history, and his communications with Del Rosa is based upon personal knowledge.

Defendant's objection to the Declaration of Olga Sencion is **OVERRULED**. Her material testimony concerning Ms. Sencion's diet and swallowing history is based upon personal knowledge as her caregiver.

III. UNDISPUTED AND DISPUTED MATERIAL FACTS

Ms. Sencion, an 81-year-old patient with significant comorbidities, including pneumonia, COVID-19, end-stage renal disease requiring dialysis, diabetes, atrial fibrillation, and a prior stroke, was admitted to Del Rosa on September 4, 2022, following hospitalization. (UFs 4-5.)

Plaintiff does not materially dispute Ms. Sencion's admission or underlying conditions. Plaintiff does, however, present evidence that Ms. Sencion had longstanding swallowing difficulties following her stroke, required a pureed or liquified diet for approximately seven years, and had previously choked on non-pureed food. Plaintiff contends Del Rosa failed to obtain this history from family and failed to perform a swallowing evaluation or dysphagia screening despite multiple risk factors for aspiration. (AFs 1-21.)

Defendant contends Ms. Sencion was appropriately assessed upon admission, required only partial assistance with feeding, and was placed on a physician-ordered no-salt-added renal, mechanical-soft diet with thin liquids, with no documented swallowing problem before September 6. (UFs 6-12, 21-27.)

Plaintiff disputes the adequacy of that assessment and care planning. Plaintiff also presents evidence that Del Rosa failed to provide or document appropriate care for Ms. Sencion's active COVID-19 infection, including evidence that a COVID-monitoring order was not entered until September 8, two days after her death. (AFs 9-26.)

Plaintiff further disputes that Del Rosa appropriately assessed Ms. Sencion's dietary needs or complied with her physician's diet order. Plaintiff presents evidence that no dietary assessment or profile card was completed, that Defendant produced no meal-intake or feeding records documenting what Ms. Sencion was served or consumed, and that the September 6 meal may not have complied with the ordered mechanical-soft and renal diet. Plaintiff also presents testimony concerning whether the chicken served should have been ground rather than chopped and whether the meal presented a choking or aspiration risk. Plaintiff contends CNA Olivia Hall

had not reviewed Ms. Sencion's diet order and lacked recent training in proper feeding techniques. (AFs 27-48.)

On September 6, 2022, Ms. Sencion developed difficulty swallowing while being fed. Defendant asserts that staff repositioned her, prepared to perform the Heimlich maneuver, suctioned her, administered oxygen, initiated CPR when she became unresponsive, and transported her to the hospital. Defendant further asserts that Ms. Sencion expelled food from her mouth and that no airway obstruction was observed upon intubation. (UFs 13-20.)

Plaintiff disputes the adequacy of that response and relies upon testimony concerning the choking event, interventions that were and were not performed, Defendant's choking-response policy, and staff training. (AFs 49-54.)

Causation is sharply disputed. Defendant relies upon Dr. Steinke's opinion that Ms. Sencion did not die from choking and that no act or omission by Del Rosa caused or contributed to her death. Plaintiff relies upon Barnes's contrary opinions concerning dysphagia-related airway compromise, together with the death certificate, AMR records, and St. Bernardine records documenting aspiration, dysphagia, choking, and a foreign body in the respiratory tract. (AFs 55-57.)

Plaintiff also identifies alleged deficiencies and inconsistencies in Del Rosa's post-incident documentation, including differences between Barrios's original and subsequent entries, statements concerning whether paramedics restored a pulse, later documentation reflecting no swallowing disorder or difficulty, and alleged gaps in meal, medication, and skilled-charting records. (AFs 58-71.)

Finally, Plaintiff presents evidence concerning Director of Nursing Tina Arteaga's authority over staffing, policy compliance, hiring, discipline, and training; CNA staffing levels on September 6; and a CDPH deficiency issued approximately one month before Ms. Sencion's death concerning compliance with abuse-reporting procedures. (AFs 72-74.)

IV. LEGAL STANDARD

Summary judgment is appropriate only where the papers submitted demonstrate that there is no triable issue as to any material fact and the moving party is entitled to judgment as a matter of law.

A defendant moving for summary judgment bears the initial burden of showing that one or more elements of the plaintiff's cause of action cannot be established or that there is a complete defense. Once that burden is met, the burden shifts to the plaintiff to demonstrate the existence of a triable issue of material fact.

The Court does not weigh competing evidence or resolve credibility disputes on summary judgment. Rather, the evidence must be viewed in the light most favorable to the nonmoving party, and all reasonable inferences must be drawn in that party's favor.

With those principles in mind, the Court addresses each issue presented by the motion.

V. DISCUSSION

1. First Cause of Action — Elder Abuse and Neglect

The abuse of an elder or dependent adult under the Elder Abuse and Dependent Adult Civil Protection Act ("Act") includes neglect. (Welf. & Inst. Code, § 15610.07, subd. (a)(1).)

A neglect elder abuse claim requires: (1) that the defendant had care or custody of the plaintiff, (2) that the plaintiff was an elder or dependent adult while in the defendant's care or custody, (3) that the defendant failed to use the degree of care that a reasonable person in the same situation would have used, (4) that the defendant's conduct harmed the plaintiff, and (5) that the defendant's conduct was a substantial factor in causing the plaintiff's harm. (CACI 3103; see also *Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, 406-07.)

Neglect is not merely the failure to provide the knowledge, skill, and care possessed by other nurses of a hospital; rather, it concerns the failure to provide basic needs and comfort to a dependent adult. (*Delaney v. Baker* (1999) 20 Cal.4th 23, 34.)

Neglect may include failure to assist with personal hygiene or provide food, clothing, or shelter; failure to provide medical care for physical and mental health needs; failure to protect against

health and safety hazards; and failure to prevent malnutrition or dehydration. (Welf. & Inst. Code, § 15610.57, subd. (b)(1)-(4).)

There is no dispute that Ms. Sencion was an elder and dependent adult residing in a skilled nursing facility and receiving custodial care.

A. Neglect

The principal issue is whether the evidence, viewed in Plaintiff's favor, permits a finding that Del Rosa failed to provide basic custodial care, as opposed to merely providing care that Plaintiff contends fell below the professional standard of care.

Defendant relies upon evidence that Ms. Sencion was assessed upon admission, placed on a physician-ordered mechanical-soft diet, and monitored during her stay. (UFs 6-12, 21-27.)

Defendant further relies upon evidence that, after Ms. Sencion exhibited difficulty swallowing on September 6, staff repositioned her, prepared to perform the Heimlich maneuver, suctioned her, administered oxygen, and initiated CPR when she became unresponsive. (UFs 13-20.)

Defendant also presents evidence that Ms. Sencion expelled food from her mouth and that no airway obstruction was observed upon intubation. (UFs 18-20.)

Dr. Steinke opines that Del Rosa's conduct did not fall below the applicable standard of care and did not cause or contribute to Ms. Sencion's death, and that her death resulted from cardiopulmonary arrest rather than choking. (Steinke Decl. ¶¶ 28, 30.)

Plaintiff, however, presents evidence supporting a materially different account of the care provided.

Plaintiff's expert, George Barnes, identifies multiple pre-existing aspiration and dysphagia risk factors, including prior stroke, recurrent pneumonia, cardiopulmonary disease, malnutrition, cognitive impairment, and feeding dependence, yet no speech-language pathology consultation or swallowing evaluation was ordered. (Barnes Decl. ¶ 36.)

Plaintiff also presents evidence that Ms. Sencion had required a fully pureed diet for years and had previously choked on non-pureed food, but that Del Rosa allegedly failed to obtain this information from family upon admission. (Olga Sencion Decl. ¶¶ 6-7, 9; Ruben Sencion Decl. ¶¶ 6-7, 9-10.) Barnes further opines that, given Ms. Sencion's swallowing and choking risk factors,

Del Rosa should have obtained an SLP consultation or performed a swallowing evaluation or screening. (Barnes Decl. ¶¶ 36, 44(a).)

This evidence is sufficient to create a triable issue. A reasonable trier of fact could conclude that the alleged failures were not limited to a disagreement concerning a professional medical judgment but instead involved the provision of basic custodial necessities to a highly dependent resident—including safe feeding, appropriate monitoring, and protection from a known health and safety hazard.

Plaintiff additionally relies upon evidence concerning alleged failures in feeding assistance and staff training (AFs 42-48), the response to the choking event (AFs 49-54), and the absence of adequate documentation concerning Ms. Sencion's meals and care.

Defendant relies upon *Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, and *Cochrum v. Costa Victoria Healthcare, LLC* (2018) 25 Cal.App.5th 1034. Plaintiff distinguishes *Carter* as a pleading-stage case and *Cochrum* as involving a resident who received care for 19 days, underwent a bedside swallowing evaluation, and was periodically monitored while eating.

The Court need not determine at this stage whether Plaintiff will ultimately prove elder neglect. The question is whether the evidence presents a triable issue. Plaintiff's evidence concerns multiple alleged failures involving assessment, diet, feeding, training, monitoring, and emergency response during Ms. Sencion's admission. Those allegations, if proven, could support a finding that Del Rosa failed to provide basic custodial care.

Accordingly, the Court finds a triable issue of material fact as to whether Defendant's conduct constituted neglect under the Act.

B. Mandatory Reporter Argument

Defendant argues that no mandated reporter identified or reported suspected abuse or neglect and contends that the absence of a report demonstrates that no elder abuse occurred.

The argument is not dispositive. The reporting statute is triggered by a "reasonable suspicion" standard. (Welf. & Inst. Code, § 15630, subd. (b)(1).) That standard does not require proof that

abuse or neglect actually occurred, nor does it determine whether the underlying conduct ultimately constituted actionable neglect.

The statutory reporting obligation is therefore distinct from the elements of liability under the Elder Abuse Act. Whether a mandated reporter possessed reasonable suspicion sufficient to trigger a reporting obligation is different from whether the evidence establishes that Defendant failed to provide necessary care.

The absence of a report may be considered with other evidence, but it does not establish as a matter of law that neglect did not occur.

C. Conclusion as to Elder Abuse

The evidence presents two materially different factual accounts. Defendant presents evidence of appropriate care and a non-choking cardiac death. Plaintiff presents evidence of multiple alleged failures in basic custodial care involving a highly vulnerable, dependent resident with significant swallowing and aspiration risk.

Because a reasonable trier of fact could find that Defendant failed to provide necessary care and that such conduct caused harm, summary adjudication of the first cause of action is not warranted.

Thus, the Court DENIES summary adjudication as to the first cause of action.

2. Second Cause of Action — Statutory Violations of the Patient Bill of Rights

Health & Safety Code section 1430, subdivision (b), provides that a “current or former resident or patient, or the legal representative, personal representative, or successor in interest ... may bring a civil action against the licensee of a facility who violates any rights of the resident or patient as set forth in Section 72527 or 73523 of Title 22 of the California Code of Regulations, or any other right provided for by federal or state law or regulation.”

Defendant contends Plaintiff has not presented competent evidence establishing a violation of a protected patient right.

The argument depends largely upon Defendant’s position that the underlying care was appropriate. As discussed above, however, triable issues of material fact exist concerning the

adequacy of Ms. Sencion's assessment, diet, feeding assistance, monitoring, emergency response, and documentation.

Plaintiff identifies alleged violations arising from that disputed care, including rights relating to participation in care planning, receipt of ordered services, a safe environment, freedom from neglect, and accurate records.

On this record, Defendant has not established as a matter of law that Plaintiff cannot prove a violation of a protected statutory or regulatory right.

The Court therefore DENIES summary adjudication as to the second cause of action.

3. Third Cause of Action — Wrongful Death

Defendant contends the wrongful death claim fails because no act or omission by Del Rosa was a substantial factor in causing Ms. Sencion's death.

The elements of a medical malpractice claim are: “(1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional's negligence.” [Citation.]” (*Hanson v. Grode* (1999) 76 Cal.App.4th 601, 606.)

In medical malpractice actions, the standard of skill, knowledge, and care prevailing in a medical community is ordinarily a matter within the knowledge of experts. Whether the standard of care has been breached generally must be established by expert testimony unless the medical issue is within the common knowledge of laypersons. (*Jambazian v. Borden* (1994) 25 Cal.App.4th 836, 844; *Landeros v. Flood* (1976) 17 Cal.3d 399, 410.)

Expert testimony is ordinarily required to establish the prevailing standard of care and whether the health care provider's conduct complied with that standard. (*Starr v. Mooslin* (1971) 14 Cal.App.3d 988, 999.) Where a defendant supports summary judgment with competent expert testimony and the plaintiff fails to submit opposing expert testimony on a matter requiring expert opinion, summary judgment may be appropriate. (*Jambazian, supra*, 25 Cal.App.4th at 850.)

Plaintiff must present competent expert testimony establishing, to a reasonable medical probability, that the defendant's acts or omissions were a substantial factor in causing the injury. (See *Bromme v. Pavitt* (1992) 5 Cal.App.4th 1487, 1492-1493.)

Here, Defendant has met its initial burden by presenting Dr. Steinke's opinion that no act or omission by Del Rosa was a substantial factor in causing or contributing to Ms. Sencion's death. (Steinke Decl. ¶ 28.) Defendant also presents evidence that Ms. Sencion developed difficulty swallowing, after which staff repositioned her, prepared to perform the Heimlich maneuver, suctioned her, administered oxygen, and initiated CPR when she became unresponsive. (UFs 13-20.) Defendant contends that Ms. Sencion expelled food from her mouth, had no airway obstruction upon intubation, and did not choke. (UFs 18-20.) Defendant instead attributes her death to cardiopulmonary arrest unrelated to choking. (Steinke Decl. ¶ 30.)

Plaintiff, however, has submitted competing expert testimony and other evidence sufficient to create a triable issue.

Barnes opines that Del Rosa failed to properly assess Ms. Sencion's swallowing function, implement appropriate dysphagia precautions, and ensure safe feeding practices despite known risk factors for choking. (Barnes Decl. ¶¶ 4, 36.) He further opines that Del Rosa failed to obtain an appropriate speech-language pathology evaluation and failed adequately to monitor and respond to signs of swallowing distress, which Barnes contends constituted departures from accepted standards for dysphagia management in a skilled nursing setting. (*Id.* at ¶¶ 4, 36.) Barnes also identifies alleged deficiencies in caregiver training, feeding assistance, and dietary precautions. (*Id.* at ¶¶ 38, 44.)

Defendant argues Barnes is not a physician and therefore cannot offer competent opinions concerning medical issues. The argument is not persuasive as to subjects within Barnes's demonstrated expertise.

Barnes is a licensed speech language pathologist and board certified specialist in swallowing and swallowing disorders with clinical experience involving dysphagia and medically complex patients. (Barnes Decl. ¶¶ 6-15.) His opinions concerning swallowing function, choking and

aspiration risk, feeding safety, airway compromise related to dysphagia, and the need for speech-language pathology evaluation fall within that expertise. (*Id.* at ¶¶ 4, 34-36.)

The Court does not rely upon Barnes for opinions outside his demonstrated expertise. He is not qualified to opine on unrelated cardiac pathology or independently determine a cardiac cause of death. He may, however, offer opinions within his area of expertise concerning whether Ms. Sencion's clinical presentation was consistent with dysphagia-related airway compromise and whether the alleged departures in swallowing and dysphagia management caused or contributed to choking during the September 6 meal. (*Id.* at ¶¶ 4, 34-35.)

The competing expert opinions create a genuine factual dispute concerning both breach and causation. Those disputes cannot properly be resolved on summary judgment.

The Court therefore DENIES summary adjudication as to the third cause of action.

4. Punitive Damages

Civil Code section 3294, subdivision (b), requires evidence that an officer, director, or managing agent of a corporate defendant authorized, ratified, or had advance knowledge of the alleged wrongful conduct before punitive damages may be imposed against the corporation.

A managing agent is an employee who exercises substantial discretionary authority over decisions that ultimately determine corporate policy. (*White v. Ultramar, Inc.* (1999) 21 Cal.4th 563, 577.)

Ratification requires actual knowledge of the wrongful conduct and its outrageous nature, followed by approval or adoption of that conduct. (*College Hospital Inc. v. Superior Court* (1994) 8 Cal.4th 704, 726.)

In determining summary adjudication of punitive damages, the Court considers the evidence with the clear and convincing burden of proof in mind. (*Basich v. Allstate Ins. Co.* (2001) 87 Cal.App.4th 1112, 1119.)

Plaintiff now presents evidence concerning Director of Nursing Tina Arteaga's authority that was not before the Court in the original opposition. Defendant's job description assigns the Director of Nursing responsibility for facility staffing levels, policy development and compliance, hiring, discipline, and staff in-service training. (AF 72.)

Viewed in Plaintiff's favor, this evidence is sufficient to create a triable issue concerning whether Arteaga exercised substantial discretionary authority over significant aspects of Del Rosa's operations and therefore qualified as a managing agent. (See *White v. Ultramar, Inc.* (1999) 21 Cal.4th 563, 577.)

Plaintiff also relies upon post-incident documentation that was part of the original record. Plaintiff previously identified evidence that Arteaga documented the family's report of prior choking incidents and the ER physician's statement that choking caused Ms. Sencion's death but thereafter verified an MDS reflecting "no swallowing disorder" and later signed documentation reflecting no swallowing difficulty or choking. (AFs 65-68.)

Plaintiff also identifies discrepancies between post-incident nursing documentation and contemporaneous AMR and hospital records. (AFs 58-64.) The supplemented record now contains additional evidence concerning Arteaga's authority and testimony bearing upon the preparation and verification of the documentation.

Taken together and viewed in Plaintiff's favor, the evidence permits a reasonable inference that Arteaga had actual knowledge of circumstances surrounding the choking event and thereafter approved or adopted documentation inconsistent with that knowledge.

The Court does not find that the evidence establishes intentional falsification or ratification as an undisputed fact. Nor does the Court resolve the competing inferences concerning the documentation. Those are matters for the jury.

The question at this stage is whether Plaintiff has produced sufficient evidence to create a triable issue when the clear and convincing standard is considered. The Court concludes that Plaintiff has done so.

The Court therefore DENIES summary adjudication as to Plaintiff's prayer for punitive damages.

VI. RULING

Based on the foregoing analysis, the Court rules as follows:

- 1. Defendant Del Rosa Villaidence Opco, LLC's Motion for Summary Judgment is DENIED.**

2. **Defendant's alternative Motion for Summary Adjudication is DENIED as to the First Cause of Action for Elder Abuse and Neglect.** Triable issues of material fact exist concerning whether Defendant failed to provide necessary custodial care to Ms. Sencion. (See UFs 4-27; Steinke Decl. ¶¶ 10-28, 30; AFs 1-57; Barnes Am. Decl. ¶¶ 4, 34-37; Ruben Sencion Decl. ¶¶ 5-10; Olga Sencion Decl. ¶¶ 3-10; Apodaca Decl. ¶¶ 4-6, 11-13, 16-17, 19, Exhs. B-E, H-J, R-S.)
3. **Defendant's Motion for Summary Adjudication is DENIED as to the Second Cause of Action for Statutory Violations of the Patient Bill of Rights.** Triable issues exist concerning the care, diet, feeding, monitoring, and documentation provided to Ms. Sencion and the resulting alleged violations of her statutory and regulatory rights.
4. **Defendant's Motion for Summary Adjudication is DENIED as to the Third Cause of Action for Wrongful Death.** The parties have presented conflicting evidence and expert opinions concerning the applicable standard of care, breach, and whether Defendant's conduct was a substantial factor in causing or contributing to Ms. Sencion's death.
5. **Defendant's Motion for Summary Adjudication is DENIED as to Plaintiff's prayer for punitive damages.** Viewed in Plaintiff's favor and with the clear and convincing burden in mind, the supplemented record creates a triable issue concerning whether Director of Nursing Tina Arteaga was a managing agent and knowingly ratified wrongful conduct committed with oppression, fraud, or malice. (See AFs 58-68, 72; Apodaca Decl. ¶¶ 4, 12-13, 16-17, 26, Exhs. B, E, H-I, S, V.)
6. **Plaintiff's First Amended Evidentiary Objections Nos. 1-3 to the Declaration of Lyndsay Sklenka and the medical records submitted therewith are OVERRULED.**
7. **Plaintiff's First Amended Evidentiary Objections Nos. 1-7 and 9 to the Declaration of Dr. Gary W. Steinke are OVERRULED.**
8. **Plaintiff's First Amended Evidentiary Objections Nos. 8 and 10 to the Declaration of Dr. Gary W. Steinke are SUSTAINED.**
9. **Defendant's Evidentiary Objections Nos. 1, 3-15, 17-18, 20, and 23 to the First Amended Declaration of George Barnes, MS, CCC-SLP, BCS-S are OVERRULED.**

10. Defendant's Evidentiary Objections Nos. 2 and 24 to the First Amended Declaration of George Barnes are SUSTAINED.

11. Defendant's Evidentiary Objection No. 16 to the First Amended Declaration of George Barnes is SUSTAINED IN PART. The objection is sustained as to: "Ms. Olivia Hall's admitted lack of training since 1998 is an egregious violation of these CMS and DHHS requirements" and "This reveals an institutional disregard for the essential training standards required to protect resident health and safety at Del Rosa Villa—a failure originating from the highest levels of management." The objection is otherwise overruled.

12. Defendant's Evidentiary Objection No. 19 to the First Amended Declaration of George Barnes is SUSTAINED IN PART. The objection is sustained as to the phrase "making it a regulatory standard" and the statements: "Either Mrs. Sencion's swallowing issues came up during these discussions and it was never documented and never addressed or the discussion never took place. In both of these scenarios, it shows the team at Del Rosa Villa acted below the standard of care." The objection is otherwise overruled.

13. Defendant's Evidentiary Objection No. 21 to the First Amended Declaration of George Barnes is SUSTAINED IN PART. The objection is sustained as to the phrase "seemingly to change the way the incident is perceived." The objection is otherwise overruled.

14. Defendant's Evidentiary Objection No. 22 to the First Amended Declaration of George Barnes is SUSTAINED IN PART. The objection is sustained as to: "This underscores a facility culture that prioritized minimal training over individual patient safety." The objection is otherwise overruled.

15. Defendant's Evidentiary Objection No. 25 to the First Amended Declaration of George Barnes is SUSTAINED IN PART. The objection is sustained as to: "Ms. Sencion was also not supposed to be provided with potatoes, being on a renal diet" and "The inclusion of at least two food items inconsistent with her dietary orders strongly suggests

that Ms. Sencion was served the wrong tray for the lunch meal during which she choked.”

The objection is otherwise overruled.

16. Defendants’ previously filed objections to the Declarations of Ruben Sencion and Olga Sencion are OVERRULED.

The Court’s rulings above are without prejudice to appropriate evidentiary objections at trial based upon the evidence and foundation actually presented at that time.